Nursing Essentials

Priority: Maternal infection and newborn sepsis can worsen quickly. Escalate concerns and avoid delays in ordered treatment.

FOCUS 1

Recognize

- **Consider infection:** maternal fever, fetal tachycardia, uterine tenderness, or foul-smelling fluid; findings vary.
- **Risk rises with:** prolonged membrane rupture and ascending infection. A single symptom does not establish the diagnosis.
- **Trend together:** temperature, maternal heart rate, fetal tracing, rupture history, and the overall clinical picture.

FOCUS 2

Treat

- **Notify and reassess:** maternal status, fetal status, perfusion, and signs of sepsis.
- **Support treatment:** ordered antibiotics, fluids, fever management, and cultures when indicated; cultures must not delay urgent antibiotics.
- **Birth planning:** coordinate with the obstetric team. Infection alone is not an automatic indication for cesarean birth.

FOCUS 3

Observe baby

- **Subtle signs matter:** poor feeding, lethargy, temperature instability, respiratory distress, apnea, or abnormal perfusion.
- **No fever required:** a normal temperature does not exclude serious newborn infection.
- **Risk assessment:** maternal history, gestational age, examination, and the local early-onset sepsis pathway guide evaluation.

Newborn nursing actions

Make it stick: Tachycardia can reflect several problems. Connect it with temperature, hydration, fetal status, and perfusion rather than labeling it in isolation.

Focus	What to know	Nursing application
Escalate instability	support airway, breathing, circulation, temperature, and glucose monitoring as indicated.	Support stabilization
Prepare	cultures and other ordered tests; give antibiotics promptly according to the neonatal plan.	Prepare ordered tests
Watch the trend	respiratory effort, oxygen needs, feeding tolerance, urine output, and perfusion.	Trend the response

RED FLAGS · ESCALATE PROMPTLY

Escalate maternal or newborn instability, poor perfusion, altered responsiveness, apnea, increasing respiratory distress, or concern for sepsis.